Nursing SOAP Note - Respiratory Imaging Follow-up

Case RNP-7A9C | Source-bounded documentation based on attached handoff and radiology case packet

Patient	55-year-old female
Reason for note	Progressive shortness of breath and cough after chest imaging review
Documentation boundary	No vital signs, oxygen saturation values, laboratory results, medication list, allergy list, or provider treatment orders were included in the source materials.

S - Subjective

- Patient-reported concern available in the handoff: progressive shortness of breath and cough.
- No additional symptom descriptors, duration details, fever status, chest pain status, baseline respiratory history, or home medication information were provided in the attached materials.

O - Objective

Authoritative chest imaging findings:

- Moderate-sized right pleural effusion which freely layers on the right-side-down decubitus view.
- Hazy opacification in the right lung base; report language favors a combination of layering pleural fluid and atelectasis.
- No pneumothorax.
- Mild enlargement of the cardiopericardial silhouette.
- Median sternotomy changes.

No objective bedside measurements such as respiratory rate, oxygen saturation, temperature, lung sounds, laboratory results, intake/output, or response to interventions were present in the supplied materials.

A - Assessment

- Imaging-supported respiratory concern: moderate right simple pleural effusion with right basilar hazy opacity/atelectatic component.
- Symptom relevance: pleural fluid around the right lung can reduce lung expansion, especially at the right base, and may contribute to progressive shortness of breath and cough. Imaging alone does not prove this is the sole cause of symptoms.
- No pneumothorax is reported on imaging.
- Mild cardiopericardial silhouette enlargement and median sternotomy changes are noted on imaging; the available materials do not establish the patient's cardiac history, baseline heart size, or clinical significance of these findings.

P - Plan / Follow-up Needs

- Promptly notify or confirm provider awareness of the symptomatic, moderate right pleural effusion and associated right basilar opacity/atelectatic component for clinical correlation and treatment decisions.
- Continue focused respiratory nursing assessment within facility policy and provider orders, including work of breathing, ability to speak comfortably, cough pattern, breath sounds if assessed, positioning tolerance, and oxygenation measurements if obtained.
- Position for breathing comfort, such as upright or head-of-bed elevation as tolerated, while awaiting provider-directed next steps.
- Use the patient education summary to explain that the fluid is around the outside of the lung and can limit expansion; reinforce that the provider will decide next steps after correlating imaging with the bedside exam and available clinical data.
- Escalate urgently for suddenly worse breathing, inability to speak full sentences, chest pain, blue lips or face, confusion, fainting, or other signs of acute distress.

Unsupported items intentionally not added: diagnoses beyond the imaging impression, vital signs, oxygen values, laboratory values, medications, allergies, comorbidities, procedure recommendations, or provider treatment orders.